

Any cardiovascular adverse events are unlikely with topical hydrocortisone therapy; however, prolonged prolific use may result in a transient hypertension as a result of fluid retention.
Long term corticosteroid use has resulted in Benign Intracranial Hypertension, with most reports occurring in children.
Corticosteroids have documented effects on serum lipids, including increased total cholesterol, increased low density lipoproteins and increased triglyceride levels.
Topical and systemic corticosteroid therapy has been implicated in posterior sub capsular cataract formation, elevated intraocular pressure, optic nerve damage and papilloedema.
Cataracts, although primarily reported with systemic corticosteroid use, have been reported with use of topical preparations.
Hydrocortisone has least potential for causing glaucoma, but reports have demonstrated this complication in patients using topical preparations on the face.

OVERDOSAGE AND TREATMENT:
Acute ingestion or accidental poisoning, even in massive doses, is rarely a clinical problem. Treatment should be symptomatic and supportive. Excessive chronic exposure results in adverse systemic effects. In such cases the use of topical corticosteroid should be discontinued, with the consideration to tapering the dose. Emesis or activated charcoal is not usually indicated unless multiple ingestion is suspected. Support the patient as necessary and treat symptomatically.

EFFECTS ON ABILITY TO DRIVE AND OPERATE MACHINERY:
Not known.

STORAGE:
Store at a temperature below 30°C. Store in original packaging.

DOSAGE FORM:
Topical cream.

PACKAGING AVAILABLE:
15g, 30g and 50g aluminium tubes with white HDPE screwcap.

MANUFACTURER:
Mepro Pharmaceuticals Pvt. Ltd.
Unit II, Q Road, Phase – IV, GIDC, Wadhwan,
Surendranagar, Gujarat 363035, INDIA

PRODUCT REGISTRATION HOLDER:
Synercam (M) Sdn Bhd,
A-17-7, BLOCK A, JAYA ONE,
NO. 72A, JALAN PROFESOR DIRAJA UNGKU AZIZ,
SEKSYEN 13, 46200 PETALING JAYA MALAYSIA.

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MECORT

Hydrocortisone Acetate Cream 1% w/w

PRODUCT DESCRIPTION:
Soft smooth, white cream, with a faint characteristic odour chlorocresol; free from extraneous matter.

PHARMACODYNAMICS:
Pharmacotherapeutic category: Corticosteroids - anti-inflammatory, antipruritic and vasoconstrictor actions.

ATC Code: D07AA02
The activity of the drugs is thought to result at least in part from binding with a steroid receptor. Corticosteroids decrease inflammation by stabilising leukocyte lysosomal membranes, preventing release of destructive acid hydrolyses from leukocytes; inhibiting macrophage accumulation in inflamed areas; reducing leukocyte adhesion to capillary endothelium; reducing capillary wall permeability and oedema formation; decreasing complement components; antagonizing histamine activity and release of kinin from substrates; reducing fibroblast proliferation, collagen deposition, and subsequent scar tissue formation; and possibly by other mechanisms as yet unknown.

PHARMACOKINETICS:
The rate and extent of hydrocortisone absorption through the skin varies among individual patients.
Following topical application of a corticosteroid to most areas of normal skin, only minimal amounts of the lipophilic drug partitions into the predominantly aqueous dermo-epidermal layer (viable epidermis + dermis) and subsequently the systemic circulation.
Absorption is, however, markedly increased when the skin has lost its keratin layer or the rate limiting properties of the stratum corneum. Physical disruption of the stratum corneum, inflammation and/or disease of the epidermal barrier (e.g. psoriasis, eczema) may result in increased absorption.
Hydrocortisone is absorbed to a greater degree from the skin behind and around the ear region, scrotum, axilla, eyelid, face and scalp than forearm, knees, elbow, palm and sole. Prolonged absorption persists even after the area of application has been washed, possibly because the drug is retained in the stratum corneum and/or the dermo-epidermal layer.
Hydrocortisone is metabolised in the liver and most tissues to biologically inactive compounds, mainly glucuronides and sulphates. These are excreted in urine, mainly conjugated as glucuronides, together with a very small proportion of unchanged hydrocortisone. Some of the topical corticosteroids and their metabolites are also excreted in the bile.

Children are at a greater risk of systemic absorption of topical steroids due to higher permeation properties of the skin and increased surface area to body mass ratio.
INDICATION:
Indicated for the relief of the inflammatory and pruritic manifestations of corticosteroid-responsive dermatoses.
RECOMMENDED DOSAGE:
Apply sparingly to affected area 2-3 times daily until condition improve, and then reduce frequency.

ROUTE OF ADMINISTRATION:
Topical
CONTRAINDICATIONS:
Tuberculous and fungal conditions of the skin, acute Herpes simplex, vaccinia, varicella and all viral infections.
• Hypersensitivity to any component of the cream or ointment.
• Not to be used in the eye.
• Topical steroids should not be used in patients with markedly impaired circulation since skin ulceration has occurred in these patients following the use of corticosteroids.
• Topical steroid should be used with caution and occlusive dressings should not be used in patients with primary skin infections.
WARNING AND PRECAUTIONS:
FOR EXTERNAL USE ONLY AVOID CONTACT WITH EYES.
If irritation develops Hydrocortisone Acetate 1% w/w cream should be discontinued and alternative therapy instituted.
Prolonged and heavy doses of hydrocortisone may have an immunosuppressant effect and thus decrease resistance to infection as well as mask signs of it. If infection of the skin is present suitable antifungal or antibacterial agents should be used first. Any occlusive dressings should be discontinued. If the infection does not respond promptly to therapy, corticosteroid therapy should be discontinued until the infection has been controlled.
Where very large areas are treated for long periods (e.g. atopic dermatitis), the possibility of systemic absorption exists, particularly if an occlusive dressing is applied. Prolonged use of large quantities of topical corticosteroids may also result in atrophic striae or acne eruptions. Due to the possible systemic absorption of topical steroids, there may be a need for periodic evaluation of hypothalamo-pituitary-adrenal (HPA)-axis suppression by using the urinary free cortisol test or the corticotrophin stimulation test. If the HPA-axis suppression is evident, withdrawal should be attempted, and the frequency of application reduced.
Manifestations of adrenal suppression in children include retardation of linear growth, delayed weight gain, low plasma cortisol concentrations and lack of response to corticotrophin stimulation.
Manifestations of intracranial hypertension include bulging fontanelles, headache, and bilateral papilloedema. Topical corticosteroid therapy in children should be limited to the minimum amount necessary for therapeutic efficacy; chronic topical corticosteroid therapy may interfere with growth and development. Parents should be advised not to use tight-fitting diapers or plastic pants on a child being treated in the diaper area, since such garments may constitute occlusive dressings.
Children have a greater surface to mass ratio than adults, and so may be at greater risk of adverse events from increased dosages.
Any corticosteroid therapy tends to elevate blood glucose levels in diabetic patients, and this should be monitored during treatment.
Corticosteroids should be used cautiously in patients with non-specific ulcerative colitis, diverticulitis, colon abscess or other pyogenic infection, colon obstruction, or extensive fistulas and sinus tracts, fresh intestinal anastomoses, active or latent peptic ulcers, renal insufficiency, hypertension, osteoporosis and myasthenia gravis.
Topical corticosteroids should be used with caution in the management of psoriasis, as exacerbation of the disease or pustular psoriasis may occur during or on withdrawal of topical corticosteroid therapy.
Use in Children:
Long term therapy in infants should be avoided as adrenal suppression may occur.

Carcinogenicity/Mutagenicity:
There is no reliable evidence of induction of carcinogenicity or mutagenicity in humans by hydrocortisone.
Effects on laboratory tests:
The use of corticosteroids has been shown to result in falsely elevated measurements of serum digoxin concentrations, but this occurred at dosages far greater than those likely to be delivered by topical therapy.
INTERACTIONS WITH OTHER MEDICAMENTS:
There are many potential drug interactions with hydrocortisone; however most are quite unlikely with topical therapy, occurring mainly with prolonged or over-use.
Carbamazepine, phenytoin and rifampicin all induce hepatic enzymes and thus lead to increased metabolism of hydrocortisone.
Concomitant use of diuretics, which also deplete potassium ion concentration in the blood, may cause hypokalaemia.
All corticosteroids antagonise the effects of neuromuscular blocking agents, such as vecuronium.
Oral contraceptives have been shown to prolong the half-life of hydrocortisone and thus potentiate its anti-inflammatory effects.
PREGNANCY AND LACTATION:
Pregnancy
Applications of large amounts of topical corticosteroids, especially the more potent ones over prolonged periods may cause foetal abnormalities. Therefore, topical corticosteroids should be used in pregnancy only when the potential benefits justify the possible risks to the foetus.
Lactation
It is not known whether topical corticosteroids are distributed into milk. However, systemic corticosteroids are distributed into milk. Topical corticosteroids should be used with caution in nursing mothers.
SIDE EFFECTS:
An itching or burning sensation may be experienced when using this medication. Topical corticosteroids may cause adverse dermatological effects. Adverse dermatological effects are most likely to occur in intertriginous and facial areas. Local adverse corticosteroid effects occur most frequently with occlusive dressings, especially with prolonged therapy, and may require discontinuation of the dressings. Atrophy of the epidermis, subcutaneous tissue and dermal collagen; drying and cracking or tightening of the skin may occur. Epidermal thinning, telangiectasia, increased fragility of cutaneous blood vessels, purpura and atrophic striae are also reported.
Other adverse dermatologic effects of topical corticosteroids include acne form eruption, vesiculation, irritation, pruritis, hypertrichosis, rosacea-like eruptions on the face, erythema, hyperesthesia, perioral dermatitis, burning or stinging sensation, folliculitis, and hypo pigmentation.
Topically applied steroids are generally non-sensitising, but allergic contact dermatitis may occur rarely.
Topical corticosteroids should be used with caution in the management of psoriasis.
Topical corticosteroids should also be used with caution in patients with impaired T cell function or in those patients receiving other immunosuppressive therapy. The result of this impairment may be the activation of latent infection or exacerbation of intercurrent infections.

Artwork Details	
Product: Mecort Cream	Colour: 1 Colour
Country / language: English	Artwork Prepared by: Amit Rathod
Component: Insert	Initiated by: Pratapbhai
Artwork Code: -	Checked by:
Dimension: L- 190 x H- 130 mm (Open Size)	Approved by:
Blister Size: -	
Note: Font Size 6.1 pt	

Printing Details
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Note: